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FRAUD ANALYTICS
 PATTERN DETECTION REPORT

FA-01
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 FA-02
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 FA-03
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 FA-04

Rule-Based Signal Detection

CLASSIFICATION RESTRICTED	REPORT DATE 21 May 2026	PATTERNS SCREENED 4 Fraud Signals	METHODOLOGY Rule-Based Detection
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Airawat Research Foundation
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 ECHS Fraud Analytics Unit

About This Analysis

This report presents the findings of ECHS Fraud Analytics patterns FA-01 through FA-04 — a systematic screening exercise applied to ECHS claims data. The analysis uses four independent rule-based checks, each designed to detect a specific billing pattern associated with fraud or irregular conduct.

Each check operates independently. A beneficiary or hospital may be flagged by one check or by several simultaneously. Cases appearing in multiple checks are the highest investigation priority, as each flag represents a separate dimension of anomaly unlikely to arise by coincidence.

NOTE: This report is produced by an automated screening system. Every flagged case is an investigative lead, not a confirmed finding. A qualified auditor must review each case before any action is taken.

Patterns Covered

- FA-01 — The 'Double Dip': A hospital submits the exact same claim (same patient, same day, same billed amount) two or more times to receive duplicate reimbursements.
- FA-02 — 'Ghost Clones': The same patient identity appears at two different physical hospital locations on the exact same calendar date, indicating identity sharing or ECHS card lending.
- FA-03 — Synthetic Identities: Multiple beneficiary profiles share the same Government UID (Aadhaar), indicating fake profiles may have been created using fragments of real identity data.
- FA-04 — The 'Revolving Door': A hospital repeatedly discharges and readmits the same patient within days to collect new base admission fees, bypassing continuous-stay billing audits.

Combined Overview — All Patterns

Pattern	Flagged Records	Unique Patients	Financial Exposure	Severity
FA-01 — Double Dip (Exact Duplicates)	218,317 claims	Top 100 groups shown	₹18.34 Cr	Critical
FA-02 — Ghost Clones (Simultaneous Admissions)	Top 100 pairs shown	96 patients	Data Unavailable*	Critical
FA-03 — Synthetic Identities (Shared UIDs)	83,545 claims	6,737 profiles	₹166.96 Cr	Critical
FA-04 — Revolving Door (Unbundling)	Top 100 pairs shown	100 patients	Data Unavailable*	High

* Financial exposure for FA-02 and FA-04 could not be retrieved at this time due to an unstable VPN connection during data extraction. These figures will be updated in the next reporting cycle.

Key Findings — Aggregate Metrics

2,18,317 Duplicate Claims (FA-01) Full dataset — all duplicates	96+ Simultaneous Pairs (FA-02) Unique patients — top 100 shown	6,737+ Profiles on Shared UIDs (FA-03) Full dataset — all shared UIDs	73,975+ Revolving-Door Readmissions (FA-04) Across 40+ hospitals — top 100 shown
₹18.34 Cr Exposure — FA-01 (Double Dip) Full dataset overpayment	₹166.96 Cr Exposure — FA-03 (Synthetic IDs) Full dataset — all shared UID claims	₹185.30 Cr Combined Quantified Exposure FA-02 & FA-04 pending VPN fix	4 Total Patterns Detected Independent fraud signals

FA-01 — The 'Double Dip' (Exact Duplicate Claims)

Pattern Description

A hospital or intermediary submits the exact same claim — identical patient ID, admission date, and billed amount — two or more times. Like ordering one pizza and being charged five times for it, the system records each submission as a separate payable event unless a deduplication check is in place.

Signal Statistics

The financial figures below are computed across the full dataset. The flagged cases table shows the top 100 groups by duplicate count — additional groups exist beyond those shown.

Metric	Value	Notes
Duplicate claim groups in dataset	100+	Top 100 shown here; full set not bounded
Total duplicate submissions (all data)	2,18,317	Excludes the first (original) submission
Total financial overpayment (all data)	₹18,33,81,677	Computed across full dataset
Highest duplicate count (top group)	547	Patient CM000002580629 at p.fatesahi
Earliest case in top 100	2013-01-13	Patient KC0046959 — hospital p.kollam

Flagged Cases — Top 100 by Duplicate Count

Patient ID	Hospital ID	Admission Date	Dup. Count	Billed Amount	Status
CM000002580629	p.fatesahi	2022-07-27	547	₹512	● Critical
MU000002699039	pol.1928	2024-08-17	476	₹0	● Critical
DL1000007590557	p.lodhird1	2024-12-04	349	₹2,998	● Critical
HY000000136855	p.nello1	2024-07-24	329	₹3,413	● Critical
KL000003494970	nbnc2012	2026-02-21	307	₹0	● Critical
000009122940	p.chapra	2025-07-16	257	₹4,404	● High
DL1000005286568	p.shakurb	2025-08-02	130	₹0	● High
TR000005523638	muthmed	2026-03-11	88	₹0	● Medium
KC0046959	p.kollam	2013-01-13	87	₹0	● Medium
TR000002099834	muthmed	2026-03-05	84	₹0	● Medium

Severity guide: ● Critical — 200+ duplicates · ● High — 75–199 · ● Medium — up to 74

FA-02 — 'Ghost Clones' (Simultaneous Admissions)

Pattern Description

The same patient identity appears at two geographically separate hospitals on the exact same calendar date. Like claiming to have attended two weddings in different cities on the same day — physically impossible. This flags either identity fraud (card lending/sharing) or deliberate data-entry manipulation at one of the facilities.

Signal Statistics

The table below shows the top 100 simultaneous admission pairs ordered by date. The actual number of such pairs across the full dataset is expected to be higher.

Metric	Value	Notes
Simultaneous admission pairs shown	100	Top 100 by date — more pairs exist
Unique patient IDs in shown sample	96	4 patients appear in multiple pairs
Most recent flagged date	2026-03-31	Multiple patients — last day in dataset
Financial exposure	N/A	Unavailable — VPN issue during extraction

NOTE: Financial Impact — Unavailable: The monetary exposure for FA-02 could not be computed in this reporting cycle due to an unstable VPN connection during data extraction from the claims payment database. This figure will be included in the updated report.

Flagged Cases — Top 100 by Date (Most Recent First)

Patient ID	Patient Name	Hospital A	Hospital B	Date	Status
AM000003716610	JAIPAL SINGH	echsfhm	guruhari	2026-03-31	● Critical
KO000003086951	C.K. GOPI	aims1978	dayatcr	2026-03-31	● Critical
NE000007285783	RABINDRA THAPA	hos.5750	hos.6217	2026-03-31	● Critical
DL1000005726658	BRAJ MOHAN	hos.6852	QRGFBD@D1	2026-03-31	● Critical
000000618264	SATYAPAL YADAV	kailashb	parashos	2026-03-30	● Critical
DL1000007992211	OM PARKASH	mdrc_yadav	dranildc	2026-03-30	● Critical
JB000002201147	DARSHAN SINGH	hos.5698	9926857210	2026-03-30	● High
DL2000007770888	KHEM SINGH	hos.5140	hos.4710	2026-03-30	● High
CM000007972340	NAFA SINGH	echsfhm	EDENHOSP	2026-03-30	● High
TR000000016838	RAJAN K	dmkksghmc	9633647879	2026-03-30	● High

FA-03 — Synthetic Identities (Shared Government UIDs)

Pattern Description

Multiple distinct beneficiary profiles share the exact same Government UID (Aadhaar number). Legitimate data-entry errors account for a small fraction of such cases; however, bulk sharing of a single UID across dozens or hundreds of profiles is a strong indicator that synthetic (fabricated) identities were created to funnel fraudulent claims.

Particularly alarming are clusters where the UID is blank ('NA'), clearly invalid ('-1'), or a known dummy value ('999999999999') — these indicate deliberate bypass of identity verification controls.

Signal Statistics

All financial figures below are computed across the full dataset of shared-UID claims. The flagged cases table shows the top 100 UID clusters by profile count.

Metric	Value	Notes
Distinct shared UID clusters shown	100+	Top 100 shown; full set not bounded
Total profiles across shown clusters	6,737	Distinct beneficiary records in top 100
Total claims from shared UIDs (all data)	83,545	Computed across full dataset
Total financial exposure (all data)	₹1,66,95,96,865	All claims from shared-UID profiles
Largest UID cluster	4,979 profiles	UID = 'NA' — blank/missing Aadhaar
Known dummy UID (999999999999)	15 profiles	Deliberate bypass of UID validation
Invalid UID value (-1)	9 profiles	System-generated placeholder value

Flagged Cases — Top 100 UID Clusters by Profile Count

Government UID	Profile Count	Sample Names Associated
NA (Blank)	4,979	SULAKHAN SINGH, PILLA RAMANAYYA, BHUVANESWARI A, RAM KALA DHUL, SHIVDEV SINGH ...
837434937138	153	K LEELAVATHI KOVILAMMA, PALAM PARAMBATH RAVINDRAN, P VIJAYAN, NARAYANAN P ...
219065383566	126	ARJU SAHA, MILI SAHA, ANUSHKA SAHA, MANI RAJ SUBBA, BINU CHHETRI ...
509086157454	105	MUKESH BASNET, POONAM CHAKRAVORTY, YUGAL KISHOR JHA, CHANDRA PRADHAN ...
685687166927	95	SUBHRA BASAK, SUSHANT MAJUMDAR, RAMA SHANKER RAY, PURNIMA DAS ...
332363646133	91	RANU CHAUDHURI, JHARNA CHOUDHURY, AJAY KUMAR CHOUDHURY, TAPASHI RUDRA ...
999999999999	15	USHA TIKOO, PREM LATA, JYOTHSNA IPPILI, MADHU, SUSHIL KUMAR SHARMA ... (dummy UID)

-1 (Invalid)	9	ROOP LAL, OSHMEET KAUR GILL, SRIPAT YADAV, P KAMALAKSHY, TARA DEVI TAMANG ...
123456789412	8	AJIT SINGH LAMBA, NARAYANAMMA, KAMALA FRANKLIN, KOSHILAYA DEVI ...
944212113505	34	GOODISELA SANKARAPPA, S MAHABOOB BASHA, M NAGALAKSHMI, T VENKATARAMI REDDY ...

FA-04 — The 'Revolving Door' (Hospital Unbundling / Readmission Fraud)

Pattern Description

A hospital repeatedly discharges and readmits the same patient within a very short window — sometimes the same day — solely to generate a new base admission fee for each episode. Like a restaurant charging a full cover charge every time a diner gets up to use the restroom and returns. This pattern bypasses continuous-stay billing audits and artificially inflates episode counts.

Signal Statistics

The table below shows the top 100 patient–hospital pairs by total readmission count. The actual number of affected pairs across the full dataset is expected to be higher.

Metric	Value	Notes
Patient–hospital pairs shown	100+	Top 100 by readmission count — more exist
Unique hospitals in shown sample	40	Multiple patients per hospital is common
Total readmissions in shown sample	73,975	Across the top 100 pairs
Highest single-patient readmission count	1,278	RANJEETA GURUNG at kailash1
Pairs spanning 10+ years	Several	Indicates systematic long-running pattern
Financial exposure	N/A	Unavailable — VPN issue during extraction

NOTE: Financial Impact — Unavailable: The monetary exposure for FA-04 could not be computed in this reporting cycle due to an unstable VPN connection during data extraction. This figure will be included in the updated report.

Flagged Cases — Top 100 by Readmission Count

Hospital ID	Patient Name	Re-admissions	First Visit	Latest Visit
kailash1	RANJEETA GURUNG	1,278	2012-04-04	2022-01-17
kailashrc2	B B SRIVASTAVA	1,187	2014-05-24	2021-07-19
kailashrc2	SHALINI	1,114	2014-10-04	2021-10-04
artemis1	SITA SOHARU	1,076	2013-11-13	2021-09-23
kailashrc2	SUNITA YADAV	1,066	2014-05-22	2021-07-21
artemis1	SHEO PAL	993	2014-08-23	2021-04-19
artemis1	SAVITRI	992	2013-12-06	2020-09-12
kailashrc2	B B SRIVASTAVA (New ID)	941	2021-05-19	2026-03-29
pmch1959	PONNAMMA THOMAS	894	2019-05-14	2026-03-28
ehrcnit	SHRI RAM	893	2015-06-29	2021-06-23

Recommended Next Steps

Immediate (FA-01 & FA-03)

Suspend all claims linked to duplicate groups with 200+ submissions and all profiles sharing UID clusters larger than 50. Initiate financial recovery proceedings for the ₹185.30 Cr quantified exposure.

Priority Review — 2 Weeks (FA-02)

Cross-verify all patients flagged for simultaneous admissions. Obtain hospital discharge records and conduct beneficiary verification interviews. Compute full financial exposure once VPN access is restored.

Next Audit Cycle (FA-04)

Conduct hospital-level audits of facilities in the Revolving Door dataset, prioritising those with 700+ readmissions per patient. Compute and recover financial exposure once data pipeline is restored.

System Controls

Enforce mandatory Aadhaar de-duplication at beneficiary enrollment. Implement real-time duplicate claim detection at point of submission. Flag same-day multi-hospital admissions for automated hold pending verification.

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